

Bladder Botox (for Bladder Control)

Patient Information · Injections that calm an overactive bladder

WARWIKI

Bladder Botox treats an **overactive bladder**: sudden urges, frequent trips and leaking with urgency. Tiny amounts of botulinum toxin are injected into the bladder muscle through a thin scope to relax it. It is an option when lifestyle changes and pills have not helped enough, or when you prefer not to take pills. The effect builds over 1–2 weeks and often lasts about 6 months.

How It Works

An overactive bladder muscle squeezes when it should not. Botox **relaxes that muscle**, so you have less urgency, fewer trips and fewer leaks. It does not work right away. You will notice a change over **1–2 weeks**. When it wears off, the treatment can be repeated.

Benefits and Risks in Numbers

1 in 5

became completely dry in the main study; most others had fewer leaks

about
1 in 6

got a urine infection in the first 3 months

about
6 in 100

needed to self-catheterize for a while (higher with diabetes)

Numbers are averages from published studies. Your own risk may differ; ask your team.

What to Know Before You Decide

- **Emptying can get harder.** By relaxing the bladder, Botox can make it hard to empty fully. A few people need to pass a thin tube (self-catheterize) until it wears off. You should be willing and able to learn this if needed.
- **Urine infections** are the most common problem.
- **It is not permanent.** Most people need repeat treatments, often about every 6 months.

Learn the Terms

Overactive bladder

Urgency and frequent urination from the bladder squeezing too soon.

Botulinum toxin (Botox)

A medicine that relaxes muscle when injected.

Detrusor

The bladder muscle that is injected.

Cystoscope

The thin camera used to give the injections.

Self-catheterization

Passing a thin tube to empty the bladder. Sometimes needed for a while after Botox.

Post-void residual

Urine left in the bladder after you urinate. It is checked after the procedure.

Will it hurt? It is usually done in the office. Numbing fluid is placed in the bladder first, so most people feel only mild pressure. Some people have it in an operating room with sedation. It takes about 15 minutes, and you go home the same day.

First 1–2 days

Mild burning or a little blood in the urine.

Weeks 1–2

The effect starts. Your team may check how well you empty.

About 6 months

The effect wears off. Repeat when symptoms return, no sooner than 12 weeks.

How to Get Ready

- A **urine test** first. Botox is not given during a urine infection.
- You will likely get an **antibiotic** around the time of the procedure.
- Tell your team about any **blood thinners**. They will tell you whether to stop them. Do not stop them on your own.
- Tell your team if you have diabetes, trouble emptying, or a nerve or muscle condition.

What Happens

- 1 Numbing fluid is placed in the bladder.
- 2 A thin **cystoscope** is passed through the urethra.
- 3 Botox is injected into the bladder wall at many small spots.
- 4 The scope is removed, and you can go home.

Call the office that did your procedure if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- A weak stream, or you feel you are not emptying
- Urine like red wine, or with clots
- Burning that gets worse after a few days

Go to an emergency department if:

- You cannot urinate for several hours and your bladder feels full or painful
- You have trouble swallowing, speaking or breathing, or new muscle weakness
- You cannot reach the office and are worried

Three Things to Remember

1. Botox relaxes the bladder muscle. It works over 1–2 weeks and often lasts about 6 months.
2. A few people need to self-catheterize for a while.
3. It can be repeated when symptoms come back.